

For Malaysia

Artwork of insert "**MEDI-ALPHA 0.25 & 1 MCG**" HLM (PCL) code **RI-A014-H14-00-01**

Size : 132 x 186 mm (*F-A014-H14-14*)

(Front)

(Back)

Medi- α 0.25 / Medi- α 1

Alfacalcidol BP 0.25 mcg / 1 mcg

Medi- α 0.25

Each softgel capsule contains :
Alfacalcidol BP 0.25 micrograms

PRODUCT DESCRIPTION

3 minim, oval shaped, white, opaque soft gelatin capsule

MECHANISM OF ACTION

Alfacalcidol (1- α -hydroxyvitamin D₃) is one of the vitamin D analogues. It is rapidly converted in the liver to 1,25-dihydroxy vitamin D₃[1,25-(OH)₂D₃], the vitamin D₃ metabolite which acts as a regulator of calcium and phosphate metabolism.

Impaired endogenous production of 1,25-dihydroxy vitamin D₃ by the kidneys appears to contribute to the disturbances in mineral metabolism found in several disorders, including renal bone diseases, hypoparathyroidism and vitamin D-dependent rickets. These disorders require high doses of vitamin D for their correction.

PHARMACOKINETICS

Alfacalcidol is rapidly absorbed from the intestine (proximal or distal) and converted into the biologically active form of vitamin D₃ (1,25-dihydroxyvitamin D₃) in the liver. Peak serum level of 1,25-dihydroxy vitamin D₃ is achieved in 12 hours after a single dose of alfacalcidol. Alfacalcidol is excreted mainly in the bile and faeces with only small amounts appearing in urine.

INDICATIONS

Alfacalcidol is used for the treatment of conditions in which calcium metabolism is disturbed due to impaired 1 α -hydroxylation such as reduced renal function, in other disorders associated with vitamin D resistance, and in calcium malabsorption of osteoporosis.

These specific conditions are :

- Osteoporosis
- Renal osteodystrophy
- Hypoparathyroidism
- Hyperparathyroidism with bone disease
- Nutritional and malabsorptive rickets & osteomalacia
- Hypophosphatemic vitamin D-resistant rickets osteomalacia

DOSAGE & ADMINISTRATION

The initial dose of Alfacalcidol depends on the body weight of the individual :
Adult and children with 20 kg body weight or above : 0.5 - 1 mcg/day
Adult and children with less than 20 kg body weight : 0.05 mcg/kg/day
Capsules must be swallowed whole.

The dosage of alfacalcidol should be adjusted subsequently to avoid hypercalcaemia based on the biochemical response.

INTERACTIONS WITH OTHER MEDICAMENTS

- Increase risk of hypercalcaemia in patients taking with thiazide diuretics, calcium, or phosphate
- Some antiepileptics such as carbamazepine, phenobarbital, phenytoin, and primidone may increase vitamin D requirements but Isoniazid & Orlistat may reduce the effectiveness and absorption of vitamin D
- Some steroid such as corticosteroid may counteract the effect of vitamin D
- No other side effects associated directly with alfacalcidol therapy have been noted.

SIDE EFFECTS

Excessive intake of vitamin D leads to the development of hypercalcaemia and its associated symptoms which are vomiting, thirst, polyuria, constipation, muscle weakness, drowsiness, and confusion. In severe cases, cardiac arrhythmias

may develop. Interindividual tolerance to vitamin D varies considerably; infants and children are generally more susceptible to its toxic effects. It has been stated that vitamin D dietary supplementation may be detrimental in persons already receiving an adequate intake through their diet and exposure to sunlight.

WARNINGS

If you are patients with renal impairment or calculi, or heart disease might be at increased risk of organ damage if hypercalcaemia occurred. The patient that receiving pharmacological dose of vitamin D their plasma-calcium and plasma-phosphate concentrations should be monitored at regular intervals, especially initially or if symptoms suggest toxicity and reduce the risk of ectopic calcification.

PRECAUTIONS

- Preexisting cardiac conditions (potential exacerbation related to persistent hypercalcaemic effects during therapeutic use)
- Arteriosclerosis (potential exacerbation related to persistent hypercalcaemic effects during therapeutic use)
- Renal impairment (potential exacerbation related to persistent hypercalcaemic effects during therapeutic use)
- Liver disease (impaired absorption and hepatic 25-hydroxylation to calcitriol)
- Hyperphosphatemia (risk of metastatic calcification)
- Sarcoidosis/other granulomatous disease (potential for enhanced sensitivity to vitamin D)
- Concurrent use of calcium-containing preparations, other vitamin D containing preparations or vitamin D analogues, or thiazide diuretics (enhanced risk of hypercalcaemia)
- Concurrent use of magnesium-containing antacids (potential for hypermagnesemia)
- Concurrent use of digitalis glycosides (potential for arrhythmias related to hypercalcaemic effects of alfacalcidol)

PREGNANCY AND LACTATION

If you are pregnant or breast feeding, inform your physician prior to the commencement of therapy with this medicine.

SYMPTOMS AND TREATMENT OF OVERDOSE

Have itching of skin, dryness of mouth, bone pain, stomach pain, diarrhea and vomiting, high blood pressure, high fever and headache continuing, irregular heartbeat. If any of these effects occur, please check with your doctor immediately.

Treatment of overdose

Hypercalcaemia can be treated by dose reduction or discontinuation of the therapy. Severe hypercalcaemia may require additional treatment with a loop diuretic, intravenous fluids and corticosteroids

STORAGE

Store below 30°C in a dry place. Avoid from heat and direct light. Keep out of reach of children.

PRESENTATION :

Blister pack in carton : 30s, 50s, 60s, 100s.

Manufactured by :
MEGA LIFESCIENCES Public Company Limited
384, Pattana 3 Rd., Bangpoo Industrial Estate,
Samutprakarn 10280, Thailand

Product Registration holder :
Mega Lifesciences Sdn. Bhd.
B-28-02, The Ascent, Paradigm,
No.1, Jalan SS7/26A, Kelana Jaya,
47301 Petaling Jaya, Selangor, Malaysia.

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